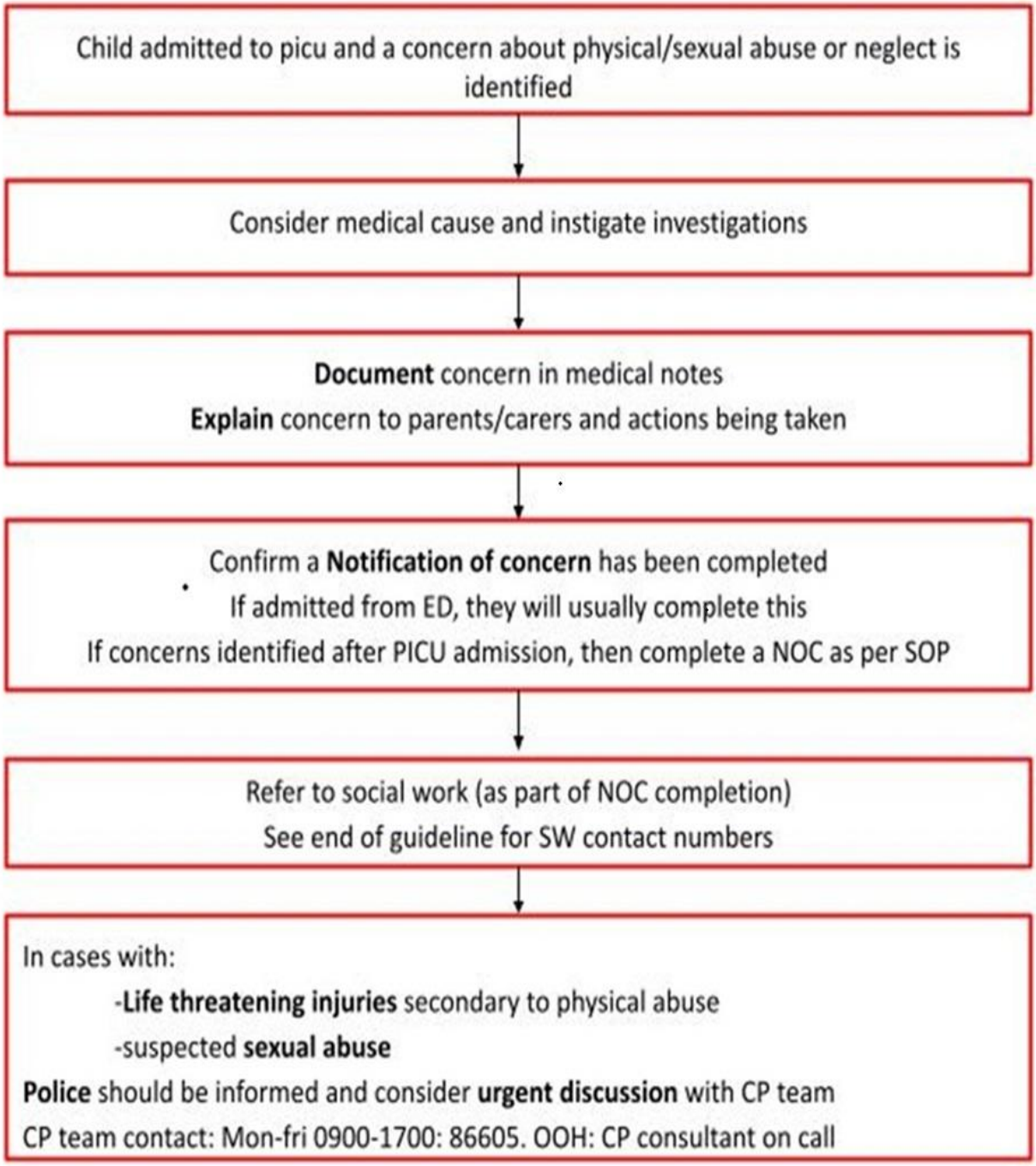
NHSGGC PUBLIC PROTECTION SERVICE

Child Protection Pathways for Patients Admitted to PICU at the Royal Hospital for Children when there is a Concern of Abuse or Neglect.

Lead Manager	**Jamie Redfern** **Director of Royal Hospital for Children**
Responsible Director	**Professor Angela Wallace, Board Nurse Director**
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Author	**Dr Mohammed Uvaise, PICM Grid Trainee.** **Dr Elliot Craven, EM trainee.** **Dr Owen Forbes, Consultant in Child Protection** **Dr Kieran Bannerman, PICM consultant**
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Replaces previous version (if applicable)	

Pathway of care for children admitted to PICU at RHC when there is a concern of Physical, Sexual Abuse or Neglect



NOC Guidance: GGC Paediatric NOC Guidance

SOP: How to complete a notification of concern form (ED SOP) | NHSGGC

Responsibilities of the PICU team:

- Prioritising Medical Safety of the patient.
- Completing a Notification of concern/Information Sharing with Social Work.
- Communicating with the GGC Child protection team.
- Family Communication.
- Updating Multi Agency teams.

Completing a NOC/ Information sharing with social work:

- The clinician who first assesses the child and identifies a concern of abuse or neglect is responsible for completing a NOC.
- In most cases, The ED team or the base hospital will have already completed a NOC and contacted local SW.
- If it is not clearly documented on a transfer letter or verbally communicated to the transfer team, the PICU team must ensure that a NOC is completed.
- With a high suspicion of physical or sexual abuse, social work must be contacted by telephone at the point of admission.
- Out-of-hours concerns can typically be actioned the next day, unless there are life-threatening injuries or concerns for the safety of other children in the household.
- Please refer to the NHS GG&C NOC guideline for further information on how to complete a notification of concern (GGC NOC guidance)
- The Following SOP provides step by step guidance on NOC completion (NOC SOP - ED).

Social work contact details:

Office hours:

- Contact the relevant patient's local area social work (See end of this guideline for the list of social work contact).
- If need assistance, can call RHC CPNA (child protection nurse advisors) on 86605.

Out of hours:

- Referrals should be made to social care direct for patients from Glasgow City (03003431505 OOH).
- Children who live outside of Glasgow City will require contact with local social work teams. (See end of this guideline for contact details)

Communicating with the GGC Child protection team:

- The GGC CP team can be contacted for advice through the below numbers
- In most cases of acute unscheduled admission, the ED team or the base hospital will have contacted SW, and the CP team will be aware of the patient.
- The CP team should always be contacted in cases with:
  - **Life threatening injuries secondary to physical abuse**
  - **Suspected sexual assault**
  - **Infants with suspected abusive head trauma**
- CP team contact details:
  - Mon – Fri 0900 – 1700: Contact CPS advice line #86605
  - OOH: Child Protection consultant on call via switchboard
- In cases with diagnostic uncertainty, a consultant-to-consultant discussion in office hours is appropriate.

Family Communication

- It is the responsibility of the PICU team to update parents on NOC completion and make them aware of what it entails.

Multi-Agency Working

- Following a child protection medical assessment, the Child Protection team will take primary responsibility for sharing of information and input into the multi-agency process.
- The attending inpatient clinicians (e.g., PICU) are responsible for providing medical updates to police and social work.
- The Child Protection Team will lead on attending multi-agency meetings and providing expert reports

- It is not the role of the PICU clinicians to provide opinion in relation to likelihood of inflicted injury and such Questions from statutory agencies should be redirected to the Child Protection team.

General Guidance

- It is the responsibility of PICU clinical staff to recognise and act on child protection concerns identified at or during the PICU admission.
- The child's medical safety and stability is always the priority.
- The PICU team's only immediate action is usually to share admission information with social work.
- If a child has a life-threatening accidental injury but there are concerns about neglect, the CP service should be contacted for case-by-case management

Child Protection Medical Assessments

- PICU clinicians are not expected to perform detailed forensic examinations for suspected physical abuse or neglect.
- If a CP concern is not clear, the PICU clinician should complete a general history and examination to determine if they are concerned and contact specialist teams (e.g. plastic surgery in case of burns)
- A Child Protection (CP) Consultant will attend the PICU to complete a medical examination for cases of physical abuse and in some cases of neglect
- This examination usually occurs during daytime hours, unless the child is not expected to survive, in these instances an examination may not be appropriate

Considering a Medical Cause

- It is important to consider and investigate potential medical causes for any presentation.
- For children under one with physical abuse concerns, refer to the "Under 1’s guidance".

Acute Sexual Assault

- Children who are victims of acute sexual assault (within the last 7 days) require immediate contact with social work and should be discussed with the child protection service.
- The Child Protection Consultant is responsible with multi agency partners for deciding whether a forensic examination is needed.
- If an examination is not performed, the CP consultant will explain the reasoning and document it in the child's record.

Intoxication / Risk-Taking Behaviour

- Children admitted due to risk-taking behaviour, such as intoxication, should be considered vulnerable.
- All these patients will require a notification of concern to be sent to social work, even if the injury is consistent with the history provided.

Interagency Referral Discussion (IRD)

- This is a discussion that takes place between health, social work and police ideally within 48 hours of a concern being raised.
- In cases of physical abuse, sexual abuse or severe neglect requiring PICU admission it is the expectation that there would always be an IRD.
- This forum is used to share information between agencies and decide whether the child protection threshold has been met.
- PICU staff may be asked to participate in the IRD process if it is felt that the information they have would be useful to this process.
- This would usually be alongside either a child protection advisor or consultant.

Social Work Area Teams Numbers:
Typically, Social Work Offices are open: -

- Monday to Thursday inclusive: 8.45 am - 4.45 pm
- Friday: 8.45 am - 3.55pm

Out with these times staff should contact Glasgow and Partners Emergency Social Work Service (Standby) on 0300-343-1505.

Glasgow city:	
Glasgow Social Care Direct	-0141 287 0555
Renfrewshire:	0300 300 1199
Paisley	
Johnstone	
Renfrew	
East Renfrewshire	0141 577 8300
Clarkston	
Barrhead	
East Dunbartonshire	0141 777 3000
Kirkintilloch	
West Dunbartonshire	
Clydebank	0141 562 8800
Dumbarton/Alexandria	01389 608080
Inverclyde	
Greenock	01475 715365
Port Glasgow	01475 715270
North Lanarkshire	
Airdrie	01236 757000

Bellshill	01698 346666
Coatbridge	01236 622100
Cumbernauld	01236 638700
Motherwell	01698348100
Wishaw	01698 348200
OOH	0800 121411